Individual Case Safety Report (ICSR)

Pharmacovigilance Case Assessment

Prepared by	**Palak Shrivas** (Drug Safety Associate)
Objective	To perform a complete pharmacovigilance assessment of the case, covering ADR identification, reporting, causality assessment and seriousness classification, and to prepare a valid ICSR for submission to the regulatory authority within the required timeline.

1. Case Summary

Report type / Source	Spontaneous, unsolicited report from a healthcare professional
Reporter	Treating physician (Dr. A.S.), General Medicine
Patient	Male, 42 years, 68 kg (initials: R.K.)
Medical history	No known drug allergy. No history of cutaneous disease, hepatic or renal disease.
Indication	Urinary tract infection

2. Suspected and Concomitant Drugs

Drug	Dose / Route / Frequency	Start	Stop	Role
Co-trimoxazole (sulfamethoxazole 800 mg + trimethoprim 160 mg)	1 tablet, oral, twice daily	04-Oct-2026	12-Oct-2026	**Suspect**
Paracetamol	500 mg, oral, as needed for fever	10-Oct-2026	12-Oct-2026	Concomitant

3. Case Narrative

A 42-year-old male patient with a urinary tract infection was prescribed co-trimoxazole 800/160 mg twice daily from 04-Oct-2026. On day 7 of therapy (10-Oct-2026) he developed fever and malaise and took paracetamol 500 mg as needed. On 11-Oct-2026 (day 8) he developed a widespread erythematous, dusky rash with painful erosions of the oral mucosa and conjunctival redness. Co-trimoxazole was stopped on 12-Oct-2026. On 13-Oct-2026 he was admitted to hospital, where examination showed skin detachment involving less than 10% of body surface area, and the clinical diagnosis of Stevens-Johnson syndrome was made; skin biopsy was consistent with the diagnosis. He was treated with intravenous fluids, systemic corticosteroids, topical wound care and antiseptic mouth wash. After withdrawal of the drug the lesions began to heal and the patient was recovering at the time of report. Rechallenge was not performed.

4. ADR Identification and Coding

Parameter	Details
Adverse drug reaction	Stevens-Johnson syndrome (fever, widespread rash, oral and ocular mucosal erosions)
Onset / Latency	11-Oct-2026 / 7 days after first dose
MedDRA coding	LLT/PT: Stevens-Johnson syndrome | SOC: Skin and subcutaneous tissue disorders
Drug coding (WHO-DD)	Sulfamethoxazole and trimethoprim (ATC: J01EE01)
Action taken with drug	Drug withdrawn
Dechallenge	Positive (lesions improved after withdrawal)
Rechallenge	Not done
Outcome	Recovering

5. ICSR Validity and Triage

Minimum criterion	Status	Information available
Identifiable patient	Met	Male, 42 years, initials R.K.
Identifiable reporter	Met	Treating physician, Dr. A.S.
Suspect drug	Met	Co-trimoxazole 800/160 mg
Adverse event	Met	Stevens-Johnson syndrome

All four validity criteria are present, so this is a **valid ICSR**. A duplicate check in the safety database (Argus / ArisG / RAVE) found no matching case, so the case is booked in as a new case.

6. Causality Assessment

A. Naranjo Scale

#	Question	Answer	Score
1	Previous conclusive reports on this reaction?	Yes	+1
2	Did the event occur after the drug was given?	Yes	+2
3	Did the reaction improve on drug discontinuation?	Yes	+1
4	Did the reaction reappear on rechallenge?	Not done	0
5	Are there alternative causes?	No	+2
6	Did it reappear with placebo?	Not done	0
7	Was the drug detected in blood at toxic levels?	Not done	0
8	Was the reaction dose-related?	No	0
9	Similar reaction to the same/similar drug before?	No	0
10	Confirmed by objective evidence?	Yes (biopsy)	+1
	Total score		**7**

Score 7 falls in the range 5 to 8, so the Naranjo category is **Probable**.

B. WHO-UMC Scale

Event has a plausible time relationship to drug intake, cannot be explained by disease or other drugs, and shows a response to withdrawal (dechallenge positive). Rechallenge was not performed. WHO-UMC category: **Probable / Likely**.

7. Seriousness, Expectedness and Severity

Seriousness	**Serious**
Seriousness criterion	Required hospitalisation (admitted 13-Oct-2026); medically significant event
Fatal / Life threatening	No
Expectedness	Expected (listed) in the co-trimoxazole prescribing information
Severity	Severe
Company comment	Event is a known serious cutaneous adverse reaction to sulfonamides. Time to onset and positive dechallenge support a causal role of co-trimoxazole. No alternative etiology identified. Drug should not be reintroduced.

8. Regulatory Timeline and Submission

Case category	Timeline	Applicable to this case
Fatal and life threatening	7 days	No
Other serious cases	15 days	**Yes** (serious, hospitalisation)
Non-serious cases	90 days	No

Workflow completed: Triage/Book-in, Data Entry, Quality Review, Medical Review. Completeness and validity checked; the case is to be submitted to the appropriate regulatory authority within **15 calendar days** of receipt of the minimum information.

9. Final Conclusion

Valid, serious, expected ICSR. Suspect drug: co-trimoxazole. Reaction: Stevens-Johnson syndrome. Causality: Probable (Naranjo 7; WHO-UMC Probable/Likely). Submission due: 15-day expedited timeline.